



Improvements: Elements of a policy solution



Improvements

Mechanisms

Procedures

Authorities

Commitments

Tactics

IMPROVEMENTS TO THE PUBLIC HEALTH SYSTEM

Objective:

Clarify with specificity the public health problem you are addressing and the desired policy improvement(s).

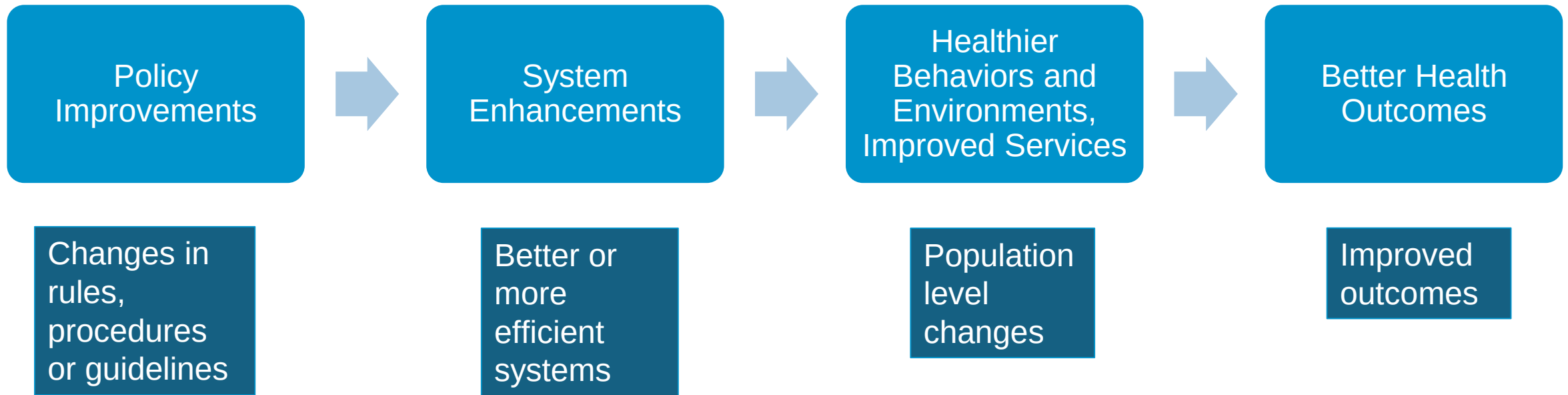
Examples:

Expanding access to specific services, improving quality of services, addressing risk factors, increasing efficiency, etc.

Key Questions:

- What is the critical root cause of the problem you are trying to address?
- How will this policy improvement work?
- Who will need to change their behavior (implementers, enforcers, users, public)?
- What are the tradeoffs in terms of health impact, political feasibility, operational feasibility, and cost?

What do we mean by “Improvements”?





Knowledge Check

Which of these is a policy improvement?

- A. Lower childhood obesity rates by 25%
- B. Increase availability of healthy foods for students
- C. Require schools to provide healthy meals to students in need
- D. Increase the number of schools with fully-staffed cafeterias

Improvements: HPV Vaccine Example

Public health problem

- High cervical cancer incidence rate
- Top cause of female cancer deaths in Africa
- Preventable by HPV vaccination

Primary cause of the problem

- Low HPV vaccine coverage (African Region average of 22% in 2023)

What **improvements** to the public health system may be needed?

WORLD HEALTH ORGANIZATION GOAL FOR HPV VACCINATION:

- 90% of girls must be fully vaccinated with HPV vaccine by age 15, by 2030
 - *Primary target:* Girls ages 9-14
 - *Vaccine schedule:* 1 dose (2 doses if HIV +ve)

The WHO recommends vaccinating boys and older women only IF feasible



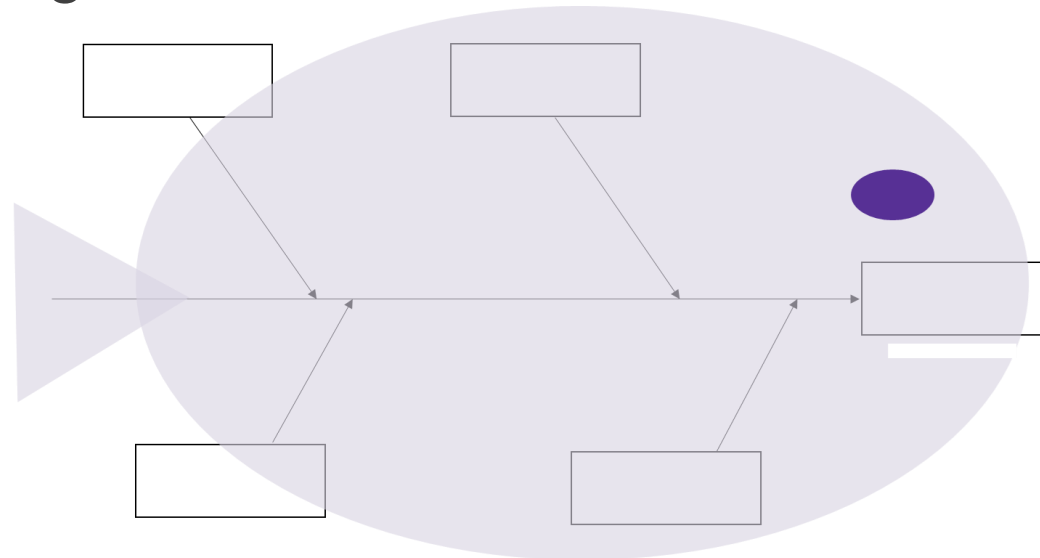
Steps for identifying Improvements

1. Target the critical root causes of our problem.
2. Address critical root causes with narrowly tailored improvements
3. Ensure completeness of our proposal by including key elements.
4. Remove ambiguity from our proposed improvements.
5. Pressure test the improvements to balance burden and public health benefits.
6. Refine and repeat as needed.

Step 1: Target the Critical Root Causes of Our Problem:

A **critical root cause** is the underlying factor or systemic issue that drives a public health problem or limits the effectiveness of current solutions.

It's the “problem behind the problem” — not just what is happening, but *why* it's happening.



HPV Example: Root causes

Problem:

Coverage below 80% in several regions, and not on track to achieve WHO target of 90% coverage of 15-year-old girls by 2030.

What improvements are needed?

Possible root causes

There is no national HPV vaccination policy or programme?

Or, there is a national HPV vaccination programme, but:

- HPV vaccination is not mandatory
- Vaccinators (health workers) lack knowledge, skills
- Insufficient vaccine supply to cover target population
- The vaccine is delivered only in schools, and does not reach girls out of school
- Insufficient financial resources to sustain implementation



Step 2: Address critical root causes with narrowly tailored improvements

Our improvements should be:

Narrow □ should focus *only* on the root cause. Shouldn't spill over into unrelated areas, which could create inefficiencies, waste, or added costs.

AND

Tailored □ the improvement is specifically designed to address the *entire* root cause without leaving any part unresolved. No gaps should remain.



Root cause

No jacket



Insufficient
solution

Short thin vest



Overly
burdensome
solution

Big bulky jacket



Narrowly
tailored
solution

Perfect fit

HPV Example: Improvements

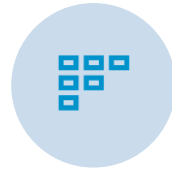
Possible root causes	Possible improvements
There is no HPV vaccination policy	Introduce a national policy
There is an HPV vaccination policy, but:	
• HPV vaccination is not mandatory	Make HPV vaccination mandatory
• Vaccinators lack knowledge, skills	Provide training & guidance for vaccinators
• Insufficient vaccine supply to cover the target population size	Micro-plans and procure sufficient HPV vaccines to cover the population
• The vaccine is delivered only in schools, and does not reach girls out of school	Deliver the vaccine also at community outreach sites and fixed/mobile clinics
• Insufficient financial resources to sustain implementation	Secure a sustainable funding stream for the HPV vaccination programme
	Etc...



Step 3: Ensure the completeness of our proposal by including key elements

1. *Have we included the important elements needed to implement our policy improvements?*
2. *Is our description complete or have we left out key requirements?*

Completeness: Common elements to include in a successful policy



Scope



Roles and
Responsibilities



Compliance and
Enforcement



Reporting and
Documentation



Resources



Timelines

HPV Example: Scope



Scope

HPV vaccination target population (who will be targeted to receive the HPV vaccine?)

Gender

- Girls only, or both girls and boys?

Age

- Target a single-age (e.g., 9, 10, or 12 year olds)

OR

- Target an age range (e.g., 9-13, 9-14 years)

Grade

- Target a school grade (e.g., grade 5 children)

HPV Example: Roles & Responsibilities



Policy stakeholders may be institutions / agencies e.g., :

Ministry of Health

Development partners, NGOs, private sector

Public & private sector health facilities

Professional associations

Community

Research and training institutions

Or individuals.....E.g., Policy on school-based HPV vaccination: who does what?

School administrators: provide space at schools for vaccine provision

District health administrators: allocate staff and resources to vaccination activities

Nurses: visit schools on vaccination days to administer vaccines, engage parents

Teachers: communicate with parents, coordinate parental consent

Parents / care-givers: provide consent

HPV Example: Compliance and Enforcement



Compliance and Enforcement

- Very few countries have established mandates for HPV vaccination – one exception is the U.S. territory of Puerto Rico

2018

DOH's official announcement HPV vaccine school-entry requirement for implementation in August for females and males aged 11–12 years.

Senate Bill 1576 as an attempt to add philosophical exemptions.

Approval of the age extension for HPV vaccination by the FDA to 45 years.

Law No. 169 of August 1: Amendment to Puerto Rico pharmacy law. Certified pharmacists can vaccinate.

HPV Example: Resources



How will resources be allocated to policy implementation?

Policy (administrative instrument): Budget allocation from National Ministry of Finance to provincial (state) departments of health for the HPV programme

Provincial departments of health must:

- Complete HPV program business plan in the prescribed format
- Include HPV vaccination indicators in provincial annual performance plans
- Use the allocated funds to strengthen provincial capacity to manage the HPV programme
- Do social mobilization to promote HPV vaccination uptake

South Africa National Treasury. 2022. Policy analysis: Roll out of the Human papillomavirus (HPV) vaccine roll out to schools.

HPV Example: Reporting & Documentation



Reporting and Documentation

Clear specification on which metrics will be reported, and how. How will the information be used?

Indicator	Definition	Baseline	2025	2026	2027	Data Source/ mean of verification	Disaggregation	Frequency
HPV vaccination coverage among girls aged 12 years	Proportion of girls aged 12 years that received HPV vaccine in the current reporting year	90%	90%	90%	90%	Immunization register, DHIS2 immunization tracker, HMIS reports, Census	Age, residence type (rural/urban)	Annually

HPV Example: Timelines



Short-term (Year 1-3)

HPV vaccine to be delivered only at public schools in selected districts



Medium-term (Year 4-8)

HPV vaccine to be delivered at public schools nationwide



Long-term (Year 8+)

HPV vaccine to be delivered at public schools nationwide plus an additional setting (outreach sites or private schools)



Step 4: Remove ambiguity from proposed improvements

1. *Are our improvements clear?*
2. *Have we defined every word precisely?*
3. *Are there unanswered questions?*



Specificity of Policy Recommendation

The health department must provide HPV vaccination to girls at schools, using an approved HPV vaccine.

Is this enough information to understand the specific policy improvements needed?



Policy Recommendation

The health department must provide HPV vaccination to girls at schools, using an approved HPV vaccine.

1. The health department
2. Must provide
3. HPV vaccination
4. Girls
5. Schools
6. Approved HPV vaccine



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Exercise:

Come up with a specific definition to each term.

Explain why you selected that definition.



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REVIEW OF EXERCISE

Define specific improvements: How will your policy improve current systems, processes and rules?



Exercise

After this material, policy advancement teams develop a list of proposed policy improvements.

Prompt given to groups:

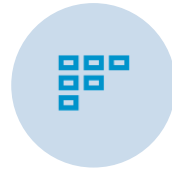
1. List the policy improvements you are recommending, that target the critical root cause identified
2. Completeness: Check that common elements are defined where relevant (BE SPECIFIC!)
3. Remove ambiguities by thinking through what questions people may ask about your improvements.
4. Discuss if the improvements are feasible and high impact
5. Note any outstanding questions or disagreements



In your groups...

- Review the assigned example from past policy advancement teams
- Discuss:
 - Is it clear what specific changes to the government's current policies, systems or procedures are being recommended?
 - Is it clear how these improvements will address the identified root cause and lead to better health outcomes?
 - Completeness/Ambiguity: Are the key elements well-defined? What questions would you ask the group to clarify further?

Completeness: Common elements to include in a successful policy



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